BASELINE → TREATMENT → FOLLOW-UP

Your tests and treatment checks

Use the individual prescription and appointment calendar. No universal tumour-marker panel or scan interval is assumed.

Check	Before starting	Early treatment	Ongoing checks	Dose hold / restart	After treatment
Blood and organ safety	Full blood count, kidney and liver tests and blood pressure; electrocardiogram (ECG) if at risk of a heart-rhythm change; international normalised ratio (blood-clotting test) (INR) if taking warfarin. Review seizures, falls, cognition, bone and cardiac health and interactions.	Blood count, kidney and liver tests every 4 weeks as clinically indicated. Blood pressure, ECG and creatinine as clinically indicated. If taking warfarin, INR weekly until a stable dose is established.	Extra tests may be needed for symptoms, dehydration or abnormal results.	Review results, recovery and previous toxicity before any restart or next dose.	Continue your prescribed androgen deprivation therapy (ADT), unless you have had surgery to remove both testes. The team reviews symptoms, blood tests, PSA and scans according to your own plan; there is no universal scan interval.
Appointments and medicines	Agree the treatment aim, dose and complete medicines list.	Take at the same time daily with or without food. Swallow the prescribed capsules or tablets whole with water; do not open, chew or crush them. If missed, take as soon as remembered that day. If the entire day is missed, resume the usual dose the following day; do not double it. Ask pharmacy if swallowing is difficult.	Keep a current dose and symptom record.	Report falls, new memory or concentration problems and troublesome fatigue. Seizure, severe headache with confusion or vision changes needs urgent help. Discuss driving safety and every new medicine with the team; interactions can change anticoagulant effects.	Keep follow-up and recovery checks even after the last dose.
Cancer-specific and specialist tests	PSA and testosterone help assess cancer activity and hormone suppression. Scans distinguish disease not seen on conventional imaging from metastatic disease; the treatment setting must be recorded.	Confirm the booked test dates with your team.	Safety tests and response tests have different purposes.	Continue your prescribed androgen deprivation therapy (ADT), unless you have had surgery to remove both testes. The team reviews symptoms, blood tests, PSA and scans according to your own plan; there is no universal scan interval.	Agree the named follow-up team and remaining checks.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. National Cancer Control Programme (NCCP) 00233 v7: official regimen

Tests in your plan

Baseline bloods and tests

WHEN

Full blood count, kidney and liver tests and blood pressure; ECG if at risk of a heart-rhythm change; INR if taking warfarin. Review seizures, falls, cognition, bone and cardiac health and interactions.

Checks organ safety and suitability before treatment.

Treatment safety monitoring

WHEN

Blood count, kidney and liver tests every 4 weeks as clinically indicated. Blood pressure, ECG and creatinine as clinically indicated. If taking warfarin, INR weekly until a stable dose is established.

Finds changes that may need supportive care, a dose hold or adjustment.

Specialist and cancer tests

If part of your plan

WHEN

PSA and testosterone help assess cancer activity and hormone suppression. Scans distinguish disease not seen on conventional imaging from metastatic disease; the treatment setting must be recorded.

Distinguishes treatment selection, hormone monitoring and cancer response.

THE WHOLE TREATMENT JOURNEY

Enzalutamide: castration-resistant prostate cancer: treatment and checks

NCCP-listed metastatic castration-resistant prostate cancer before or after docetaxel, or high-risk nonmetastatic castration-resistant prostate cancer. Your team records your exact setting.

Enzalutamide 160 mg once daily, continuously while helping and tolerated. Continue ADT unless surgically castrated. A lower dose may be prescribed for toxicity or interactions.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Early treatment	Ongoing checks	Dose hold / restart	After treatment
Blood and organ safety NCCP	Full blood count, kidney and liver tests and blood pressure; ECG if at risk of a heart-rhythm change; INR if taking warfarin. Review seizures, falls, cognition, bone and cardiac health and interactions.	Blood count, kidney and liver tests every 4 weeks as clinically indicated. Blood pressure, ECG and creatinine as clinically indicated. If taking warfarin, INR weekly until a stable dose is established.	Extra tests may be needed for symptoms, dehydration or abnormal results.	Review results, recovery and previous toxicity before any restart or next dose.	Continue your prescribed androgen deprivation therapy (ADT), unless you have had surgery to remove both testes. The team reviews symptoms, blood tests, PSA and scans according to your own plan; there is no universal scan interval.
Appointments and medicines NCCP	Agree the treatment aim, dose and complete medicines list.	Take at the same time daily with or without food. Swallow the prescribed capsules or tablets whole with water; do not open, chew or crush them. If missed, take as soon as remembered that day. If the entire day is missed, resume the usual dose the following day; do not double it. Ask pharmacy if swallowing is difficult.	Keep a current dose and symptom record.	Report falls, new memory or concentration problems and troublesome fatigue. Seizure, severe headache with confusion or vision changes needs urgent help. Discuss driving safety and every new medicine with the team; interactions can change anticoagulant effects.	Keep follow-up and recovery checks even after the last dose.
Cancer-specific and specialist tests NCCP	PSA and testosterone help assess cancer activity and hormone suppression. Scans distinguish disease not seen on conventional imaging from metastatic disease; the treatment setting must be recorded.	Confirm the booked test dates with your team.	Safety tests and response tests have different purposes.	Continue your prescribed androgen deprivation therapy (ADT), unless you have had surgery to remove both testes. The team reviews symptoms, blood tests, PSA and scans according to your own plan; there is no universal scan interval.	Agree the named follow-up team and remaining checks.

What each check is for

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Treatment and review

Once daily plus ongoing ADT

This treatment aims to slow cancer growth and help control symptoms. Your team explains the expected benefit, uncertainty and alternatives for your situation.

Ask / act: Blood count, kidney and liver tests every 4 weeks as clinically indicated. Blood pressure, ECG and creatinine as clinically indicated. If taking warfarin, INR weekly until a stable dose is established.

Sources: NCCP

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-14 · Source reconciled draft; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.51